



Location: SHERMAN OAKS HOSPITAL

PATIENT NAME : JUAREZ, CEJA JUAN LUIS

D-O-B : 07-25-1978

STUDY DATE : 07-31-2025

APPROVAL DATE : 08-01-2025

TO SPEAK TO A RADIOLOGIST: 877-939-7378

PATIENT ID : A311585

ACCESSION NO : A1015814

REFERRING PHYSICIAN : SALCIDO ADOLFO

RADIOLOGIST : ADIL MAZHAR

MRI OF RIGHT HIP

TECHNIQUE: Multiplanar, multisequence magnetic resonance imaging of the hip joint was performed in neutral position. Sequences were obtained in sagittal, coronal and axial planes and submitted for interpretation.

CLINICAL HISTORY: Pain.

FINDINGS:

Alignment : Normal.

Pincer:

Retroversion: Absent.

Over- coverage: Absent.

Os acetabuli: Absent.

Cam:

Head- neck angle: Small osseous bump at the right femoral head/neck junction likely reflects a CAM deformity that may be related to clinical femoral acetabular impingement syndrome. Incidental finding of CAM deformity is also seen in the left hip.

Fibrocystic lesion of femoral neck: None.

Dysplasia: None.

Fluid: There is minimal right hip joint effusion. Incidentally, minimal left hip joint effusion is seen.

Labrum: Normal.

Cartilage:

Femoral cartilage: Normal.

Acetabular cartilage: Normal.

Capsule and ligaments: Normal.

Muscles, tendons, and entheses:

Flexors: Normal.

Extensors: Normal.

Abductors: Normal.

Adductors: Normal.

Rotators: Normal.

Hamstrings: Normal.

Other findings: Small subcortical cysts in the bilateral femoral heads.

Other findings:

Vessels: Normal.

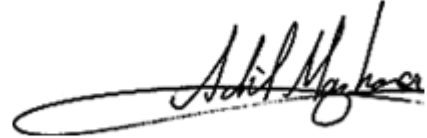
Nerves: Normal.

Soft tissues: Small fat-filled bilateral inguinal hernias.

Viscera: No abnormality seen in pelvis; No lymphadenopathy; No free fluid in the pelvis.

Impression:

1. Small osseous bump at the right femoral head/neck junction likely reflects a CAM deformity that may be related to clinical femoral acetabular impingement syndrome. Incidental finding of CAM deformity is also seen in the left hip.
2. There is minimal right hip joint effusion. Incidentally, minimal left hip joint effusion is seen.
3. Small subcortical cysts in the bilateral femoral heads.
4. Small fat-filled bilateral inguinal hernias.



ADIL MAZHAR
RADIOLOGIST

Time Finalized: 2025-08-01 09:22:48

